“Committed to Professional Excellence”

BUSINESS PLAN FOR IMPERIAL COLLEGE OF MEDICAL AND HEALTH SCIENCES — NEW COURSE EXPANSION FEASIBILITY PLAN IN THIKA TOWN, KIAMBU COUNTY

DIRECTORS’ BUSINESS PLAN | THIKA MAIN CAMPUS | NOVEMBER 2026

IMPERIAL COLLEGE OF MEDICAL AND HEALTH SCIENCES (ICMHS)

NOVEMBER 2026

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EXECUTIVE SUMMARY / SYNOPSIS

Decision sought: Determine the commercial, academic, regulatory and operational viability of introducing Mortuary Science, Optometry, Orthopedic and Trauma Medicine, Artificial Intelligence for Health and Medical Applications within an 18-month implementation horizon.

ICMHS is an existing private medical and health-sciences training institution with an established institutional identity and practical-training orientation. All six programme lines in this plan are assigned to the Thika Main Campus in Thika Town, approximately 600 metres from Thika Level 5 Hospital, subject to final capacity and approval checks. The expansion should be treated as a staged portfolio decision rather than an automatic launch. Each offering requires a separate approval gate covering curriculum, entry requirements, staffing, facilities, equipment, placement capacity, learner demand, pricing and contribution to institutional sustainability.

The 2027 fee schedule provides terms and tuition for six programme lines. At stated planned intakes, potential full-course tuition is KES 33,750,000, collected over five to eight terms. Equipment estimates total KES 20,948,815.80 provisionally; Orthopaedic and AI/Data Science costs remain unconfirmed. Adding KES 1,800,000 setup and KES 4,200,000 working-capital allowances gives a provisional capital envelope of KES 26,948,815.80. Recurring operating cost of KES 16,260,000 is an unreconciled annual source estimate. Available evidence does not support an annual profit forecast.

Mortuary Science, Optometry, Orthopedic and Trauma Medicine, Artificial Intelligence for Health and Medical Applications should proceed through a controlled stage-gate process: validate demand; confirm regulator and professional-body requirements; secure curriculum and academic leadership; confirm campus and placement capacity; obtain quotations; authorize a pilot intake; review results; and approve scale-up only if the agreed thresholds are met.

CHAPTER ONE: BUSINESS DESCRIPTION

1.1 Business Name and Brand Concept

The proposed initiative is an academic portfolio expansion by Imperial College of Medical and Health Sciences (ICMHS), not a new standalone business. It will use ICMHS governance, brand standards, academic systems, campus infrastructure and institutional support functions, subject to capacity confirmation.

The portfolio comprises Mortuary Science Level 5 and Level 6, Orthopaedic Trauma Medicine Level 5 and Level 6, Diploma in Optometry, and Artificial Intelligence and Data Science.

1.2 Business Location and Physical Infrastructure

All six programme lines are planned for the ICMHS Thika Main Campus, Thika Town, approximately 600 metres from Thika Level 5 Hospital, according to the official ICMHS campus page. The published campus profile describes modern lecture halls, science laboratories, medical skills laboratories, a digital library, computer facilities, Wi-Fi-enabled learning spaces and access to clinical attachments.

The exact plot/street address and postal address are not published on the official pages reviewed and must be confirmed by ICMHS administration. Before launch, the campus audit must document room capacity, skills-lab readiness, ICT connectivity, power resilience, sanitation, accessibility, storage, security, emergency response, staff workspaces and course-specific approval evidence.

1.3 Form of Business Ownership

The existing Private Limited Company will own and oversee the expansion. The Board or equivalent governing authority should approve the investment envelope, risk appetite and launch sequence.

Management should appoint an accountable programme sponsor, academic lead, finance lead, quality-assurance representative, facilities lead and industry/clinical liaison. Decision rights, escalation routes and monthly reporting responsibilities must be recorded before implementation.

1.4 Nature of Business and Industry Sector

The portfolio aligns with ICMHS's medical and health-sciences identity while adding a specialist technology pathway through Data Science. The opportunity is based on potential demand for practical, employability-oriented training and on the institution's ability to share academic, administrative and quality systems across programme offerings.

The opportunity remains conditional. Management must validate employer demand, competitor provision, entry requirements, qualification recognition, professional scope, placement availability and the affordability of proposed fees before committing capital.

1.5 Products and Services Offered

Programme names, levels and fees below follow the 2027 ICMHS fees circular. Confirm each regulator approval, award and entry criteria before marketing or admissions.

1.6 Vision, Mission, and Core Values

Motto: Committed to Professional Excellence

Mission: To offer transformative, medical & health training in Africa.

Vision: Offering support to your career dreams.

The expansion should be governed by ICMHS core values: Self Discipline, Accountability, Respect, Excellence, Integrity and Innovativeness. These values should appear in programme

approval, faculty selection, learner support, assessment controls, data governance and stakeholder engagement.

1.7 Business Goals and Objectives

Establish a verified feasibility case for each proposed course within the approved planning horizon.

Confirm curriculum, award, entry, faculty, facility and placement requirements before public launch.

Obtain comparable supplier quotations and replace all dummy costs with approved budgets.

Complete a controlled pilot intake with defined quality, financial and operational thresholds.

Review actual enrolment, retention, delivery cost, placement performance and learner outcomes before scale-up.

The accountable owner should convert these objectives into a signed stage-gate register with dates, evidence owners and approval status.

1.8 Justification and Commercial Viability

The expansion can be viable if incremental tuition contribution exceeds incremental delivery, staffing, compliance, placement, technology and support costs while protecting existing academic quality. Shared use of campuses and central services may reduce duplication, but programme-specific equipment and specialist staff remain material commitments.

The decision must therefore consider contribution by course, minimum viable intake, cash timing, capital payback, regulatory lead time, placement capacity and downside scenarios rather than relying on total portfolio revenue alone.

1.9 Entry and Growth Strategy

ICMHS should use a staged launch: evidence gathering; regulatory and curriculum confirmation; facility and equipment readiness; controlled marketing; pilot intake; first-term review; and scale-up decision.

Marketing should communicate only confirmed awards, entry requirements, fees, campus, duration and approval status. Course claims, employment outcomes, professional scope and placement promises must be approved by management and supported by evidence.

1.10 SWOT Analysis

CHAPTER TWO: MARKETING PLAN

2.1 Potential Customers and Market Segments

Primary demand segments include KCSE school leavers, working health-service personnel seeking progression, employers, county and private health facilities, technology-oriented applicants, sponsors and professional development partners.

Demand evidence must be collected separately for each course through enquiry records, employer interviews, competitor review, fee comparisons, open-day registrations, placement discussions and documented survey responses. No assumed demand figure should be treated as verified until the evidence register is complete.

2.2 Market Share and Projected Total Demand

Management should set a minimum viable intake, target intake and maximum safe capacity for every course. The admissions team should report enquiries, offers, acceptances, deposits, registrations, withdrawals and deferrals by course and campus.

2.3 Competitor Analysis

The relevant comparison set includes accredited medical training colleges, universities, specialist professional providers and credible online or blended alternatives offering comparable awards or competencies. Competitor names, fees, duration, entry requirements, facilities and approval status must be collected and dated before the final decision.

ICMHS should position the portfolio around verified practical training, accessible delivery, accountable learner support, employer relevance and responsible use of health technology. These are planning propositions, not confirmed competitive advantages.

2.4 Pricing Strategy and Cost Structure

The supplied figures are the current ICMHS fee schedule: fee per semester multiplied by the stated number of semesters gives the full-programme fee. Finance should still confirm payment timing, scholarships, placement costs, consumables, assessment charges, technology subscriptions and the institution's required margin. Direct course costs are intentionally left as [Costing required] until approved course-level budgets are supplied.

2.5 Advertising and Sales Promotion Strategy

Recruitment should use the ICMHS website, approved prospectus, open days, school and employer outreach, professional networks and direct enquiry follow-up. Communications must identify the course, approved award, campus, entry requirements, duration, fee basis, intake date and contact channel.

ICMHS has an active marketing team. It will lead enquiries, outreach, open days, application conversion and course-level funnel reporting. This plan adds no invented marketing budget; incremental campaigns should follow the approved annual budget.

2.6 Distribution Strategy and Channels

Delivery may combine campus teaching, skills-lab practice, supervised clinical or industry placement, computer-based work and approved blended learning. The final mix must follow the regulator-approved curriculum and each course's practical requirements.

The operating model must include admissions, timetabling, attendance, assessment, placement coordination, safeguarding, disability support, complaints handling, records management and progression monitoring.

2.7 Sales Forecast and Revenue Projections

At full planned intake and completion, six programme lines generate KES 33,750,000 tuition over their full duration. This is not Year 1 revenue or launch cash. Forecast collections by term using cohort start dates, payment schedules, arrears, discounts and withdrawals. Other charges in the fee circular remain excluded until mapped by course.

Finance should model conservative, base and upside cases for each course, including delayed launch, lower intake, fee discounts, payment arrears, staffing delays, equipment inflation and placement costs. Revenue should be recognized against confirmed registration and the institution's approved accounting policy.

CHAPTER THREE: ORGANIZATION AND MANAGEMENT PLAN

3.1 Organizational Structure / Organogram

The department will be led by one Head of Department, supported by one Course Coordinator and the trainers required for approved course delivery. The Head of Department reports to the Principal/Managing Director and is accountable for departmental performance, staffing, budget, quality and compliance. The Course Coordinator manages the timetable, learner progress, assessments, records and day-to-day course delivery. Trainers deliver theory and practical instruction, assess competence and maintain course records. Finance, Admissions and Marketing, Quality Assurance, Facilities, ICT and Clinical/Industry Liaison provide shared institutional support; they do not replace the department’s academic reporting line.

The Head of Department owns the departmental plan and reports monthly on enrolment, fee collection, staffing, teaching delivery, practical readiness, placements, quality issues and budget variance. The Course Coordinator maintains the underlying course and learner records. Finance validates financial reporting, while Quality Assurance independently monitors academic and regulatory controls.

3.2 Management Team Background

Management must demonstrate academic leadership, regulator engagement, financial control, procurement discipline, quality assurance, data protection, facilities planning, clinical or industry partnership management and change management.

Individual names and credentials should be inserted only from current HR files and verified certificates. The final plan should attach a responsibility matrix and evidence register rather than relying on assumed profiles.

3.3 Key Personnel and Job Specifications

Course-specific faculty ratios, teaching loads, professional registration and practical-supervision requirements must be confirmed from the applicable curriculum and regulator. No intake should be opened without the required staff appointed or contractually secured.

3.4 Remuneration and Staff Benefits

The earlier staffing schedule totals KES 405,000 monthly before statutory costs, benefits and additional teaching. Validate roles, teaching load, employment status and allocation by course before forecasting.

Payroll approval should require evidence of workload, qualification, registration where applicable, timetable allocation and budget availability.

3.5 Recruitment, Training, and Development

Recruitment will prioritize verified subject expertise, teaching ability, practical supervision, assessment competence and professional conduct. Induction should cover ICMHS policies, safeguarding, assessment integrity, data protection, emergency procedures, equipment use and incident reporting.

Annual development plans should address curriculum updates, simulation practice, digital teaching, quality audits, workplace feedback and regulator requirements.

3.6 Regulatory Licenses, Health Permits, and Compliance

Before launch, management must confirm the regulator, qualification owner, programme approval route, entry requirements, faculty credentials, facility standards, clinical or industry placement rules, assessment requirements, advertising permissions, data-protection obligations and occupational safety controls for each course.

The compliance register should record requirement, authority, evidence, responsible owner, submission date, expiry date, cost and status. Any unverified item is a no-launch condition.

3.7 Supporting Professional Services

The expansion will require coordinated support from legal advisers, finance, procurement, ICT, data-protection officers, facilities teams, clinical or industry partners, equipment service providers, insurers and quality-assurance personnel.

All external engagements should use written scopes, service levels, safeguarding provisions, confidentiality terms, data controls, insurance requirements and termination conditions.

CHAPTER FOUR: OPERATIONAL AND PRODUCTION PLAN

4.1 Production Facilities and Space Allocation

The proposed portfolio requires a controlled mix of classrooms, course-specific skills laboratories, simulation areas, ICT facilities, staff workspaces, stores, sanitation, accessibility features, power backup, secure records storage and placement coordination space.

The draft planning footprint is 650 square metres. This figure must be replaced after a campus audit that records usable area, timetable availability, safety requirements, equipment clearances and maximum safe occupancy by course.

4.2 Production Process Workflow

Confirm approved curriculum, timetable and teaching resources.

Admit only applicants meeting verified entry requirements.

Deliver theory, practical and simulation sessions with attendance controls.

Assess competence using approved methods and maintain moderation records.

Prepare and monitor clinical or industry placement where required.

Review attendance, progression, incidents, feedback and resource use each term.

Escalate quality or safety exceptions to the academic and management committees.

4.3 Machinery, Tools, and Equipment Sourcing

Mortuary Science and Optometry workbook estimates are KES 6,534,181.40 and KES 7,614,634.40. Orthopaedic Trauma (KES 3,600,000) and AI/Data Science (KES 3,200,000) are unconfirmed earlier-draft estimates. The KES 20,948,815.80 subtotal is provisional pending itemized quotations, VAT, delivery and installation.

4.4 Raw Material Requirements and Sourcing

The earlier schedule lists monthly allowances of KES 65,000 Mortuary, KES 90,000 Optometry, KES 110,000 Orthopaedic and KES 140,000 AI/Data Science, totalling KES 405,000. Validate quantities, supplier prices and cohort usage.

4.5 Labor Requirements and Workflow Scheduling

Timetables should protect the required balance between theory, practical sessions, simulation, assessment, placement and learner support. Shared rooms and equipment must be scheduled centrally with buffer time for setup, cleaning, maintenance and incident response.

The academic office should publish a term calendar, course workload, room allocation, faculty assignment, assessment plan and placement calendar before registration closes.

4.6 Quality Control and Waste Management

Controls should cover equipment inspection, practical-session risk assessment, PPE, infection prevention where applicable, sharps and clinical waste handling, chemical or biological exposure, data security in the Data Science lab, emergency response, incident reporting and routine internal audits.

Each course should maintain a quality file containing lesson plans, attendance, assessment records, equipment logs, consumable issues, placement feedback, complaints, incidents and corrective

actions.

4.7 Production Capacity and Manufacturing Schedule

The first intake should be limited by verified room capacity, equipment-to-learner ratios, faculty availability and placement capacity rather than by marketing targets. Management should approve a maximum safe cohort size for each course.

Preventive maintenance schedules, calibration records, software renewals, backup power tests and replacement plans should be funded through the annual operating budget and reviewed monthly.

CHAPTER FIVE: FINANCIAL PLAN

5.1 Pre-Operational Financing and Start-Up Capital

The capital schedule is a feasibility estimate. Mortuary and Optometry amounts are workbook calculations; Orthopaedic and AI/Data Science amounts remain unconfirmed. Setup and working capital are allowances. Obtain approved quotations and a monthly cash requirement before authorization.

5.2 Sources of Enterprise Capital

No funding source or financing commitment is confirmed. Finance the provisional KES 26,948,815.80 envelope only after validating quotations, funding capacity, terms, timing and repayment ability.

Finance should compare equity, retained earnings, asset finance, grants, strategic partnerships and borrowing. Any debt case must include approved terms, security, repayment schedule, covenants and sensitivity to slower enrolment.

5.3 Key Financial Assumptions

Tuition values use the 2027 fee circular and planned intakes. Course direct costs, term collection timing, shared overhead, financing and tax remain Finance inputs. Costs labelled unconfirmed must not be approved as quotations.

Every assumption should be assigned an evidence owner and status: verified, management estimate, quotation required, regulator confirmation required or not yet available.

5.4 Pro-Forma Cash Flow Statement (Three Years)

The cash model should stage equipment payments, staff onboarding, approval costs, marketing, deposits, fee collections, refunds, placements, consumables, maintenance and debt service by month. Tuition should be modelled by actual payment plan rather than assumed full collection at registration.

Management should maintain conservative, base and upside cash cases and a minimum cash reserve trigger. The current working-capital placeholder is KES 4,200,000 and must be replaced after the monthly cash schedule is approved.

5.5 Pro-Forma Income Statement (P&L)

The six lines produce KES 33,750,000 potential tuition across programme duration. The source plan estimates annual operating cost of KES 16,260,000 but does not reconcile its basis to course delivery. Comparing lifetime tuition with one year of costs would mislead. Finance should complete a term-by-term three-year model before directors assess profit, break-even, payback or debt capacity.

This is a decision-support estimate, not an approved forecast. Finance must produce course-level three-year statements after replacing intake, fee, staffing, facility, placement, consumable, tax and financing assumptions.

5.6 Pro-Forma Balance Sheet

The balance-sheet model should recognize equipment and fit-out assets, cash, receivables, prepayments, payables, deferred fee income, borrowing, institutional equity and accumulated results. Asset registers should record ownership, location, serial number, condition, depreciation policy and insurance.

The final balance sheet must reconcile to the approved cash flow, income statement, procurement schedule and financing documents.

5.7 Break-Even Analysis

Break-even must be calculated separately for each course using approved fee, direct cost, staffing, placement, facility and shared-overhead allocations. A portfolio-level figure can conceal an underperforming course.

The current model should not be approved until management confirms minimum viable intake, payment timing, course contribution, shared-cost allocation and the treatment of one-off capital expenditure.

5.8 Profitability and Financial Health Ratios

The implementation dashboard should track contribution per learner, fee collection rate, cost per registered learner, payroll-to-income ratio, operating margin, cash runway, equipment utilization, return on incremental capital and payback period.

Thresholds should be approved before launch, with corrective actions for low intake, fee arrears, cost overruns, staff vacancies, equipment downtime and placement shortfalls.

5.9 Risk Management and Contingency Plan

REFERENCES (APA 7TH EDITION)

REFERENCES

The final document should add current regulator notices, approved curricula, competitor fee evidence, supplier quotations, placement agreements, internal management accounts and the

institution's approved policies. Sources must be dated and tied to the assumptions they support.

APPENDIX I: BUSINESS IMPLEMENTATION SCHEDULE (GANTT CHART)

APPENDIX I: PROPOSED IMPLEMENTATION SCHEDULE

APPENDIX II: MACHINERY AND RAW MATERIAL SUPPLIER QUOTATIONS

APPENDIX II: EVIDENCE REGISTER

DIRECTORS' FINANCIAL REVIEW

The financial schedules distinguish verified fees from estimates. The 2027 fee circular supports the course terms and tuition. Mortuary and Optometry equipment amounts are workbook calculations; Orthopaedic and AI/Data Science amounts remain unconfirmed. Annual operating expenses, setup and working capital require Finance reconciliation.

Portfolio tuition build

This is lifetime tuition value for planned cohorts assuming full enrolment, persistence and collection. It is not annual revenue or cash at launch. Term dates, payment cadence, discounts, arrears and withdrawals must drive the three-year forecast. Other charges listed in the fees circular are excluded until mapped by course.

Operating cost reconciliation

The unitemized difference is the supplied annual OPEX less annualized payroll and consumables. Finance should identify utilities, placements, assessment, maintenance, shared services and other included costs before adopting the total.

Three-year model schedules

Income statement, cash flow and balance sheet should be linked. Current sources lack cohort start dates, collection timing, confirmed direct costs, opening balances, financing terms, depreciation policy and tax treatment. Those fields must be supplied by Finance before forecasts are treated as approved.

Revenue sensitivities (not forecasts)

Illustrations below apply simple uptake percentages to full-course tuition value. They hold price constant and exclude timing, cost, discount and collection effects.

Break-even and return measures

Use the formulae after Finance validates inputs. Do not compare whole-course tuition with one year of operating expenses; that mixes time periods and cannot establish profit.

Financial approval gates

Appendix: source and assumption register

Appendix: Director dashboard

IC PRIVATE MEDICAL, HEALTH SCIENCES AND APPLIED TECHNOLOGY EDUCATION	
Abbreviation	Full Meaning
APA	American Psychological Association
KES	Kenya shilling
P&L	Profit and loss
SWOT	Strengths, weaknesses, opportunities and threats

Programme line	Intake	Terms	Fee / term	Full tuition / learner	Cohort tuition
Mortuary Science Certificate L5	25	5	KES 28,000	KES 140,000	KES 3,500,000
Mortuary Science Diploma L6	25	8	KES 34,000	KES 272,000	KES 6,800,000
Orthopaedic Trauma Certificate L5	25	5	KES 30,000	KES 150,000	KES 3,750,000
Orthopaedic Trauma Diploma L6	25	8	KES 33,000	KES 264,000	KES 6,600,000
Optometry Diploma	25	8	KES 34,000	KES 272,000	KES 6,800,000
Artificial Intelligence and Data Science	35	6	KES 30,000	KES 180,000	KES 6,300,000
TOTAL	160				KES 33,750,000

Course	Purpose and rationale	Proposed level	Delivery model	Regulatory status
Mortuary Science	Assess demand for structured mortuary operations, anatomy, embalming, bereavement support, infection prevention and compliant mortuary practice training.	Level and curriculum to be confirmed with the relevant regulator	Skills-lab demonstrations plus supervised field/practicum placement	Approval pathway, curriculum and practice-site requirements [Confirm locally]

Course	Purpose and rationale	Proposed level	Delivery model	Regulatory status
Optometry	Assess demand for eye-care support and optometric training linked to community screening, refraction and supervised clinical practice.	Level and curriculum to be confirmed with the relevant regulator	Optometry skills laboratory plus supervised clinical placement	Approval pathway, professional-body requirements and clinical-site capacity [Confirm locally]
Orthopedic and Trauma Medicine	Assess demand for practical orthopedic, fracture-care, trauma-response and rehabilitation-support skills across health facilities and emergency-care settings.	Level and curriculum to be confirmed with the relevant regulator	Simulation-based skills training plus supervised clinical placement	Approval pathway, scope of practice and clinical placement requirements [Confirm locally]
Artificial Intelligence for Health and Medical Applications	Assess demand for applied AI, health informatics, data literacy and responsible automation skills supporting modern healthcare delivery.	Level and curriculum to be confirmed with the relevant regulator	Computer laboratory, project-based learning and industry-linked delivery	Curriculum ownership, certification route, data-protection and laboratory requirements [Confirm locally]

Area	Institutional observation	Required control
Strength	Existing ICMHS identity, governance and practical-training environment	Assign a portfolio sponsor and use existing quality systems
Strength	Potential cross-use of central services and campus infrastructure	Complete a capacity audit before allocating shared resources
Weakness	Specialist faculty, equipment and placement arrangements may be scarce	Secure verified staff and partner commitments before launch
Opportunity	Specialist health and applied-technology training may diversify income	Validate demand and competitor fees through documented research
Threat	Delayed approvals or unclear professional scope	Maintain an approval register and no-launch gate
Threat	Intake below break-even or cost escalation	Pilot conservatively and monitor course-level contribution monthly

Programme line	Planning intake	Evidence required before approval
Mortuary Science Level 5	25	Demand and placement validation
Mortuary Science Level 6	25	Demand and placement validation
Orthopaedic Trauma Medicine Level 5	25	Demand and placement validation
Orthopaedic Trauma Medicine Level 6	25	Demand and placement validation
Diploma in Optometry	25	Demand and clinical placement validation
Artificial Intelligence and Data Science	35	Demand and employer validation

Course	Fee per term	Duration (terms)	Full programme tuition	Direct cost per learner
Mortuary Science Certificate (Level 5)	KES 28,000	5	KES 140,000	[Costing required]
Mortuary Science Diploma (Level 6)	KES 34,000	8	KES 272,000	[Costing required]
Orthopaedic Trauma Medicine Certificate (Level 5)	KES 30,000	5	KES 150,000	[Costing required]
Orthopaedic Trauma Medicine Diploma (Level 6)	KES 33,000	8	KES 264,000	[Costing required]
Diploma in Optometry	KES 34,000	8	KES 272,000	[Costing required]
Artificial Intelligence and Data Science	KES 30,000	6	KES 180,000	[Costing required]

Position	Headcount	Planning cost	Accountability / qualification	
Head of Department	1	KES 80,000/month (source estimate)	Department leadership; relevant postgraduate qualification and verified experience [confirm]	Relevant postgraduate qualification and verified experience in health-sciences academic leadership [Confirm].
Course Coordinator	1	KES 60,000/month (source estimate)	Timetable, records, learner progress and assessments; relevant professional qualification and teaching experience [confirm]	Relevant professional qualification, teaching experience and regulator-recognized credentials [Confirm].
Trainers	As required by approved courses	[Costing required]	Qualified subject trainers and practical supervisors; ratios, credentials and workloads to be confirmed	Relevant technical qualification and practical laboratory experience [Confirm].
Total identified monthly salary estimate	2 named roles only	KES 140,000 plus trainer costs	Excludes statutory costs, benefits, part-time load and shared support	Diploma or degree in marketing, admissions, business or related field.

Equipment / facility requirement	Qty	Unit estimate	Total estimate	Procurement status
Mortuary science teaching laboratory: cadaveric anatomy models, embalming demonstration station, mortuary trolley, PPE and infection-control fixtures	1	KES 2,850,000	KES 2,850,000	Specialist medical-equipment suppliers [Quotation required]

Equipment / facility requirement	Qty	Unit estimate	Total estimate	Procurement status
Optometry skills laboratory: slit lamp, trial lens sets, autorefractor/keratometer, tonometer, retinoscope, visual acuity and refraction equipment	1	KES 4,200,000	KES 4,200,000	Optometric equipment suppliers [Quotation required]
Orthopedic and trauma skills lab: fracture fixation models, traction and splinting sets, orthopedic instruments, casting station and simulation manikins	1	KES 3,600,000	KES 3,600,000	Orthopedic simulation suppliers [Quotation required]
Artificial intelligence and health-informatics lab: workstation computers, GPU-capable servers, networking, power backup and secure data environment	1	KES 3,200,000	KES 3,200,000	ICT systems integrators [Quotation required]
Shared skills-lab consumables starter stock: gloves, masks, gowns, syringes, dressings, casts, lenses, disinfectants and simulation disposables	1	KES 1,250,000	KES 1,250,000	Medical consumables suppliers [Quotation required]

Input category	Planning quantity	Monthly estimate	Proposed source
Mortuary and infection-control consumables	Planning allowance for practical cohorts	KES 65,000	Licensed medical consumables supplier [Quotation required]
Optometry practice consumables and disposable diagnostic supplies	Planning allowance for practical cohorts	KES 90,000	Optometric supplier [Quotation required]
Orthopedic casting, splinting and trauma simulation consumables	Planning allowance for practical cohorts	KES 110,000	Orthopedic supplier [Quotation required]
AI lab cloud, software, data-governance and cybersecurity allowance	Monthly platform and support requirement	KES 140,000	ICT provider [Quotation required]

Investment component	Planning amount
Course-specific equipment and facilities	KES 15,100,000
Pre-operational approvals, curriculum, setup and launch preparation	KES 1,800,000
Working-capital buffer	KES 4,200,000
Indicative initial funding requirement	KES 21,100,000

Risk	Early indicator	Mitigation
Approval delay	Missing regulator evidence or unresolved curriculum issue	Maintain a dated approval register and no-advertising gate
Low enrolment	Enquiries and deposits below threshold	Pilot one course, revise outreach and defer capital commitments
Faculty shortage	Unfilled specialist teaching or supervision hours	Build a verified pool of full-time and part-time faculty
Placement shortfall	Fewer confirmed places than the cohort requires	Secure written agreements before registration
Equipment cost escalation	Quotations exceed budget	Obtain alternatives, phase procurement and retain contingency
Data or safety incident	Unauthorized access, injury or non-conformance	Apply SOPs, training, insurance, audits and incident response

Source	Link
ICMHS — Mission, Vision and Values	https://icmhs.co.ke/mission-and-values
ICMHS — Official Home Page and Academic Programmes	https://icmhs.co.ke/
ICMHS — Faculty and Leadership	https://icmhs.co.ke/faculty-and-leadership

Phase	Timing	Key deliverables	Gate
Evidence and demand validation	Months 1–2	Demand file, competitor review, employer and placement interviews	Management go/no-go
Regulatory and curriculum confirmation	Months 2–5	Approval pathway, curriculum, entry requirements and faculty criteria	Academic/regulatory gate
Design and procurement	Months 4–8	Room audit, specifications, quotations, procurement and SOPs	Investment gate
Staffing and partner readiness	Months 6–10	Appointments, contracts, placement agreements and training	Readiness gate
Controlled recruitment and pilot intake	Months 9–12	Approved communications, admissions and first cohort	Pilot gate
Review and scale-up	Months 13–18	Performance dashboard, audit, lessons learned and scale decision	Board decision

Evidence item	Owner	Status	Required before final approval
Current programme approvals and regulator correspondence	Academic/Quality	Required	Yes
Approved curricula, entry requirements and award titles	Academic	Required	Yes
Campus room, safety and equipment audit	Facilities	Required	Yes

Evidence item	Owner	Status	Required before final approval
Faculty credentials and availability	HR/Academic	Required	Yes
Clinical or industry placement agreements	Liaison office	Required	Yes
Supplier quotations and service terms	Procurement	Dummy estimates	Yes
Demand survey, enquiry and competitor fee evidence	Admissions/Marketing	Required	Yes
Course-level budget and three-year financial model	Finance	Dummy estimates	Yes

Measure	Amount	Basis / confidence
Full-course tuition value	KES 33,750,000	160 planned learners; tuition × terms
Mortuary equipment	KES 6,534,181.40	Workbook calculation including 16% VAT factor
Optometry equipment	KES 7,614,634.40	Workbook stated total; quote verification needed
Orthopaedic equipment	KES 3,600,000	UNCONFIRMED earlier-draft estimate
AI/Data Science equipment	KES 3,200,000	UNCONFIRMED earlier-draft estimate
Equipment subtotal	KES 20,948,815.80	Provisional; includes unconfirmed inputs
Setup and working capital	KES 6,000,000	KES 1.8m + KES 4.2m planning allowances
Initial capital envelope	KES 26,948,815.80	Provisional; reforecast after quotes

Programme	Intake	Fee / term	Terms	Tuition / learner	Cohort tuition
Mortuary Science L5	25	28,000	5	140,000	3,500,000
Mortuary Science L6	25	34,000	8	272,000	6,800,000
Orthopaedic Trauma L5	25	30,000	5	150,000	3,750,000
Orthopaedic Trauma L6	25	33,000	8	264,000	6,600,000
Optometry Diploma	25	34,000	8	272,000	6,800,000
AI and Data Science	35	30,000	6	180,000	6,300,000
TOTAL	160				33,750,000

Cost category	Monthly	Annualized	Status
Payroll schedule	405,000	4,860,000	Earlier plan estimate; verify scope and statutory costs
Consumables/platform schedule	405,000	4,860,000	Earlier allowance; quotes and quantities required

Cost category	Monthly	Annualized	Status
Unitemized difference to OPEX	545,000	6,540,000	Arithmetic difference only; not a new estimate
Total recurring OPEX	1,355,000	16,260,000	Source plan total; Finance reconciliation required

Schedule	Driver	Control
Tuition receipts	Course × cohort × term × fee	Reconcile learners to registration and cash receipts
Direct teaching	Faculty hours and practical sessions by course	Approved workload and cost-centre allocation
Consumables and placements	Learners × session use / placement events	Stock issue records and written partner terms
Shared costs	Room hours, staff time or headcount allocation	Document driver; prevent double counting
Capital and funding	Quoted items, tax, delivery, installation and payment milestones	Track commitment, payment and asset register
Working capital	Monthly outflow less scheduled collections	Board minimum cash reserve and escalation rule

Uptake	Equivalent learners	Lifetime tuition
50%	80	KES 16,875,000
75%	120	KES 25,312,500
100%	160	KES 33,750,000

Metric	Model calculation	Current status
Contribution / learner	Net tuition less attributable direct variable costs	Direct costs not validated by course level
Break-even intake	Fixed course costs ÷ contribution / learner	Not calculable from current evidence
Operating margin	Operating surplus ÷ recognized annual revenue	Cohort timing not modeled
Cash runway	Unrestricted cash ÷ monthly net cash outflow	Opening cash and timing unavailable
Capital payback	Cumulative incremental cash flow vs. approved CAPEX	Equipment partly unquoted

Gate	Evidence	Owner
Before procurement	Itemized specifications, comparable quotes, VAT, delivery and lifecycle cost	Finance and Procurement
Before marketing/admissions	Approved award, entry criteria, fee basis, capacity and faculty readiness	Academic and Quality
Before intake	Term cash model, minimum viable cohort, placement evidence and reserve	Directors and Finance
After first term	Collections, learner persistence, direct cost and forecast variance	Portfolio sponsor

Gate	Evidence	Owner
Before scale-up	Course contribution, quality results, capacity and capital return case	Board

Input	Value	Source / status
Fees and terms	2027 course schedule	ICMHS fees PDF
Intakes	25 per line; AI/Data Science 35	Earlier Imperial plan
Mortuary equipment	KES 6,534,181.40	OPTOMETRY.xlsx; workbook formulas
Optometry equipment	KES 7,614,634.40	OPTOMETRY.xlsx stated total
Orthopaedic equipment	KES 3,600,000	UNCONFIRMED earlier-draft estimate
AI/Data Science equipment	KES 3,200,000	UNCONFIRMED earlier-draft estimate
Recurring OPEX	KES 16,260,000/year	Earlier plan estimate; reconcile
Setup / working capital	KES 1.8m / KES 4.2m	Planning allowances; validate

Indicator	Definition	Review
Enquiry conversion	Registered learners ÷ qualified enquiries	Monthly
Collection rate	Cash received ÷ tuition due	Monthly
Course contribution	Recognized tuition less direct course costs	Each term
Budget variance	Actual + commitments less approved budget	Monthly
Placement coverage	Confirmed slots ÷ learners needing placement	Before admissions
Cash runway	Available cash ÷ monthly net outflow	Monthly